

Menopause and HRT

NICE Menopause NG23 (2024), Joint position statement on menopause management BMS/RCOG (Post Reproductive Health, 2022;0(0)1), BMS guidelines (2020) and BNF

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Guidelines & Evidence Made Simple

Symptoms (due to falling oestrogen levels)

- Hot flushes, night sweats.
- Menstrual irregularities.
- Sleep disturbance.
- Vaginal dryness.
- Urinary problems.
- Joint and muscle pains.
- Sexual difficulties (e.g. low libido).
- Mood changes.
- Cognitive disturbance.

Diagnosis and investigations

Diagnosis is made from a clinical history of typical symptoms in a woman ≥ 45 y, but remember:

- Investigate abnormal bleeding first.
- Consider differentials if symptoms atypical (e.g. alcohol, drugs, thyroid, anxiety, tumours).

Arrange FSH if:

- <45 y: 2 results >30 IU/L 4–8w apart is suggestive of early menopause, or premature if <40 y.
- >50 y and amenorrhoeic and wants to stop hormonal contraception: if 1 result >30 IU/L, she can stop contraception in 12m (the FSRH consensus is that she is likely to no longer be fertile by then).

Treatment options

Lifestyle

- Regular exercise.
- Healthy diet and BMI.
- Stop smoking.
- Reduce alcohol.
- Avoid symptom triggers e.g. spicy food.
- Reduce stress.
- Relaxation exercises.
- Sleep hygiene.

Hormone replacement therapy (HRT)

First line for:

- Vasomotor symptoms.
- Depressive symptoms with other menopausal symptoms

Beneficial for:

- Sexual function.
- Urogenital symptoms (vaginal oestrogen is first line).
- Bone mineral density.

Alternatives to HRT

- CBT (+/- HRT)
- For vasomotor symptoms, low mood, insomnia and anxiety.
- Isoflavones (soy), black cohosh
- May help flushes but preparations vary in content/safety.
- SSRIs/SNRIs
- If HRT not suitable/desired. Counsel regarding side-effects.

Contraindications to HRT: refer to menopause specialist

- History of breast cancer or oestrogen-dependent tumour.
- Untreated endometrial hyperplasia.
- Undiagnosed vaginal bleeding.
- Uncontrolled hypertension.
- Arterial thromboembolic disease.
- Current or past VTE (unless anticoagulated).
- Thrombophilic disorder or strong FH of VTE.
- Liver disease (with abnormal LFTs).

Counselling for HRT: (note contraindications above)

Condition	Risk
Breast cancer	• Small increased risk: risk greater with combined HRT than oestrogen-only HRT. • Individual risk depends on modifiable and non-modifiable risk factors. • Lifestyle factors (obesity, excess alcohol) likely to have greater impact on risk than HRT. • HRT is <i>unlikely</i> to increase risk of breast cancer in women <50y. • Risk of exposure from HRT should be counted from age 50y only. • Micronised progesterone and dydrogesterone may be safest progestogens.
VTE	• HRT increases VTE risk. Individual risk depends on other risk factors. • VTE risk generally low in early post-menopause years. • Oral preparations carry higher risk. Transdermal oestrogen appears safer as it avoids first-pass liver metabolism and doesn't activate clotting factors (not confirmed by RCTs). • Micronised progesterone appears to be the safest progestogen, but more data is needed.
Cardiovascular disease (CVD)	• CVD is the commonest cause of death in postmenopausal women. • HRT does not increase the risk of cardiovascular disease in women <65y. • HRT may be cardioprotective in younger postmenopausal women (<10y from LMP), but evidence not strong enough to recommend as primary prevention of CVD.
Stroke (CVA)	• Oral HRT increases the risk of stroke. • Transdermal preparations appear safer (observational evidence). • Risk of CVA depends on other cardiovascular risk factors but generally low in this age group.
Ovarian cancer	• Slight increased risk (based on epidemiological studies, causation cannot be inferred).
In summary: HRT use may be associated with small risks, but these are balanced by benefits of treatment in a symptomatic menopausal woman, and, overall, use of HRT is unlikely to affect life expectancy.	

Menopause and HRT: which HRT?

NICE Menopause NG23 (2024), British Menopause Society guidelines (2020) and BNF



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Types of HRT

Oestrogen-only HRT

Oestrogen is the symptom-relieving component of HRT BUT

*Unopposed oestrogen causes endometrial proliferation: women with a uterus **MUST** have progestogen to stop this.*

Sequential combined HRT

Perimenopausal women

(Endogenous hormone production will lead to irregular bleeding on a continuous regimen)
Daily oestrogen with sequential progestogen (usually in last 12–14d of pack/month) to trigger a bleed.

Continuous combined HRT

Postmenopausal women
(>12m since LMP).

Contains daily oestrogen and progestogen.

Swap from sequential to continuous combined within 5y of starting HRT or by age 54y.

Which HRT?

Does she have a uterus?

Yes

No

Does she require contraception?

(<55y, sexually active and menstruating)

Contraception NOT required if:

- ≤50y and has been amenorrhoeic for ≥2y.
- >50y and has been amenorrhoeic for ≥1y.

No

Yes

Has she been amenorrhoeic for >1y?

Yes

No

Offer continuous combined HRT:

- Continuous combined pill.
- Continuous combined patch.
- Oestrogen-only pill, patch, gel or spray with:
 - 52mg IUS.
 - Oral progestogen (options and doses p4).

Offer oestrogen-only HRT

- Pill, transdermal patch, gel or spray.
- UNLESS she has had:
 - Sub-total hysterectomy.
 - A hysterectomy due to moderate/severe endometriosis.
- These women should be started on combined HRT.

Offer any of the following HRT/contraception options:

- Oestrogen-only pill, patch, gel or spray + 52mg IUS.
- Combined hormonal contraception (if eligible).
- Sequential combined HRT (pill or patch) + progestogen-only contraception (tablet, implant, injection).
- If amenorrhoeic with progestogenic contraception: could use continuous combined HRT instead of sequential.
- If declines hormonal contraception: advise barrier methods with sequential combined HRT.
- **Stop hormonal contraception at age 55y.**

Offer sequential combined HRT:

- Sequential combined pill.
- Sequential combined patch.
- Oestrogen-only pill, patch, gel or spray with:
 - 52mg IUS.
 - Oral progestogen (options and doses p4).

Troubleshooting

• **Persistent symptoms:**

- Increase to maximum licensed dose.
- Consider switch to alternative preparation.

• **Unscheduled bleeding:** common in first 3–6m, but may indicate pathology:

- Examine and investigate as per BMS guidelines (see article on *Approaching and managing problematic bleeding on HRT*).
- Once pathology excluded, manage bleeding by reducing oestrogen, changing progestogen type or increasing progestogen dose.

• **Progestogenic side-effects:**

- Switch to alternative progestogen or consider IUS.

Review

(3m after starting and then annually)

- Discuss symptom control, side-effects and ongoing indication.
- Check BP, BMI and screening up to date.
- Reinforce lifestyle optimisation.

Stop HRT when...

- Risks outweigh benefits
 - No arbitrary limits should be placed on age or duration of use.
- Patient happy to stop.

3. HRT: formulations

(from MIMS 2025 and *The BMS Tools for Clinicians: Progestogens and endometrial protection HRT preparations and equivalent alternatives*)

Oral: usually cheapest.

Transdermal: lower VTE/CVA risk so use preferentially if >60y, overweight or migraines.

Dosing: start with low-dose preparation (and match progestogen dose if using separate formulation – see *HRT regimens and progestogen doses* for details).

If urogenital symptoms predominate: consider using vaginal oestrogens instead.

Approximate prices per month:

£ = usually <£5

££ = usually £5–10

£££ = usually >£10

Combined HRT regimens

Sequential combined

Route	Oestrogen	Progestogen	Name	Cost (per month)
Oral	Oestradiol 1–2mg	Dydrogesterone 10mg (14 tabs)	Femoston	££
	Oestradiol valerate 2mg	Norethisterone 1mg (12 tabs)	Novofem	£
	Oestradiol 1mg		Elleste Duet	£
	Oestradiol 1–2mg			
	Oestradiol 1mg (10 tabs) 2mg (12 tabs)	Norethisterone 1mg (10 tabs)	Trisequens (phasic preparation)	£
Patch	Oestradiol 50mcg/24h	Norethisterone 170mcg per 24h (2 patches/w for 14d)	Evorel Sequi	£££

Continuous combined

Route	Oestrogen	Progestogen	Name	Cost (per month)
Oral	Conjugated equine oestrogen 300mcg	Medroxyprogesterone acetate 1.5mg	Premique Low Dose (also available generically)	£
	Oestradiol 0.5–1mg	Dydrogesterone 2.5–5.0mg	Femoston Conti	££
	Oestradiol 1–2mg	Medroxyprogesterone acetate 2.5–5.0mg	Indivina	££
	Oestradiol 1mg	Norethisterone 0.5mg	Kliovance	£
	Oestradiol 2mg	Norethisterone 1mg	Elleste Duet Conti	££
			Kliofem	£
	Oestradiol 1mg	Progesterone 100mg	Bijuve	££
Patch	Oestradiol 50mcg/24h	Norethisterone 170mcg per 24h (2 patches/w)	Evorel Conti	£££
	Oestradiol 50mcg/24h	Levonorgestrel 7mcg per 24h (1 patch/w)	Femseven Conti	£££

Other HRT preparations

Route	Content	Dose	Name	Cost (per month)
Oral	Tibolone (gonadomimetic)	2.5mg	Livial	££
			Generic tibolone	£

4. HRT: formulations (continued)

(from MIMS 2025 and *The BMS Tools for Clinicians: Progestogens and endometrial protection; HRT preparations and equivalent alternatives*; BMS - management of unscheduled bleeding on hormone replacement therapy)



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Approximate prices per month:

£ = usually <£5

££ = usually £5–10

£££ = usually >£10

Oestrogen-only formulations and progestogens

Oestrogen-only (only if no uterus/Mirena in place)

Route	Oestrogen	Name	Cost (per month)
Oral	Conjugated equine oestrogen 300mcg/625mcg/1.25mg	Generic preparations	£
	Oestradiol 1–2mg	Elleste Solo	£
		Progynova	£
		Zumenon	£
Patch	Oestradiol 25/50/75/100mcg per 24h	Estraderm MX	££
		Evorel	£
	Oestradiol 25/37.5/50/75/100mcg per 24h	Estradot	££
	Oestradiol 50/75/100mcg per 24h	Femseven	££
	Oestradiol 50/100mcg per 24h	Progynova TS	££
Gel	Oestradiol gel (0.06%) 1–4 pumps (1.25–3.0mg) per 24h	Oestrigel 80g pump	£–££
	Oestradiol gel (0.1%) 0.5–1.5mg per 24h	Sandrena 0.5/1.0mg sachets	££
Spray	Oestradiol 1.53mg/metred dose 1–3 sprays per 24h	Lenzetto 8.1ml (56 actuations)	£–££

Progestogen/progesterone-only (to be used with oestrogen-only HRT if has uterus)

Route	Progestogen	Dose		Name	Cost
		Sequential (take daily for 12–14d of a cycle)	Continuous (take everyday)		
Oral	Medroxy-progesterone acetate	10mg with low/moderate-dose oestrogen 20mg with high-dose oestrogen (off-licence)	2.5mg with low-dose oestrogen 5.0mg with moderate-dose oestrogen 10mg with high-dose oestrogen (off-licence)	Provera is the only brand available, but is not licensed for HRT (although endorsed by BMS)	£–££
Oral	Norethisterone	5mg irrespective of oestrogen dose	1mg with low-dose oestrogen (this dose is not available so the POP Noriday (350mcg) may be used at a dose of 3 tablets a day as an off-licence alternative) 5mg for moderate to high-dose oestrogen	Generic norethisterone: not licensed for endometrial protection, but endorsed by BMS	££
Oral/ vaginal (vaginal use is off-licence)	Micronised progesterone	200mg with low/moderate-dose oestrogen 300mg with high-dose oestrogen (off-licence) (Note: licensed dose is 10–12d/cycle, but BMS recommends 12–14d)	100mg daily with low/moderate-dose oestrogen 200mg daily with high-dose oestrogen (off-licence) (Note: licensed dose is from d1–26 of a cycle, but BMS recommends everyday)	Utrogestan	££
				Gepretix	£
				Generic preparation	£
Intra-uterine	Levonorgestrel 52mcg	20mcg/24h		Mirena (licensed) Levosert or Benilexa (off-licence)	£

We make every effort to ensure the information in these pages is accurate and correct at the date of publication, but it is of necessity of a brief and general nature. The information presented herein should not replace your own good clinical judgement, or be regarded as a substitute for taking professional advice in appropriate circumstances. In particular, we suggest you carefully consider the specific facts, circumstances and medical history of any patient, and recommendations of the relevant regulatory authorities. We also suggest that you check drug doses, potential side-effects and interactions with the British National Formulary. Save insofar as any such liability cannot be excluded at law, we do not accept any liability for loss of any type caused by reliance on the information in these pages. July 2025. For full references see the relevant Red Whale articles.